



File No.: EXP202210504

RESOLUTION OF RIGHTS PROCEDURE

After carrying out the procedural actions provided for in Title VIII of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter LOPDGDD), the following FACTS have been established:

FIRST: On September 29, 2022, a complaint was received by this Agency presented by Mr. A.A.A. (hereinafter, the complainant) against the NAVARRE HEALTH SERVICE - OSASUNBIDEA (hereinafter, the respondent) for not being properly attended to regarding his access to the medical history of his deceased wife, as he did not receive a response.

After instructing the corresponding procedure, the Director of the Spanish Agency for the Protection of Data decided not to admit the complaint for processing after analyzing the allegations presented by the respondent following the transfer of the complaint, in which it was considered that the right had been attended to.

The complainant, dissatisfied with the resolution issued, filed a motion for reconsideration against it. In the instruction of said motion for reconsideration, and before issuing a resolution, the reasons for it were communicated to the respondent so that they could formulate any allegations they deemed appropriate, without any response being received by this Agency.

On June 23, 2023, the Director resolved to uphold the motion for reconsideration filed and to admit the complaint for processing.

SECOND: The documentation in the file was transferred to the respondent for them to present any allegations they deemed appropriate, and they indicated that "(...) It is notified that the Navarre Health Service sent the complete medical history to the address specified by the applicant via certified mail on June 16, 2023."

THIRD: After examining the document submitted by the respondent, it is transferred to the complainant, so that, within ten working days, they can formulate any allegations they consider appropriate, indicating, in summary, that "(...) although we have received a good part of the requested medical history (...)" it is not complete in accordance with the provisions of Article 15.2 of the Patient Autonomy Law, as some of the documents that are part of the minimum content of the medical history are missing, in this case, "k) the operating room report (...)"

"(...) the Navarre Health Service states that the request was fulfilled, but analyzing the extensive but insufficient documentation, the absence of documents such as: the evolution reports from specialists and the follow-up that their MAP conducted on Ms. B.B.B. was observed again, and very particularly, due to the importance of such documents, the surgical protocols of the procedures performed (...) Embolization with coils and stent of a basilar artery aneurysm (...) Thrombectomy and stent implantation to achieve greater opening of the same in segment P1."

FIFTH: The responsible party was granted a hearing, and there is no record that they have submitted any written allegations.

LEGAL GROUNDS

I

Competence

In accordance with the powers granted to each supervisory authority by Article 58.2 of Regulation (EU) 2016/679 (General Data Protection Regulation, hereinafter GDPR), and as established in Articles 47, 48.1, and 64.1 of the LOPDGDD, the Director of the Spanish Agency for the Protection of Data is competent to initiate and resolve this procedure.

Furthermore, Article 63.2 of the LOPDGDD states that: "The procedures processed by the Spanish Agency for the Protection of Data will be governed by the provisions of Regulation (EU) 2016/679, this organic law, the regulatory provisions issued for its development, and, as long as they do not contradict them, subsidiarily by the general rules on administrative procedures."

II

Preliminary Issues

In accordance with the provisions of Article 55 of the GDPR, the Spanish Agency for the Protection of Data is competent to perform the functions assigned to it in Article 57, including the enforcement of the Regulation and promoting awareness among controllers and processors regarding their obligations, as well as addressing complaints filed by a data subject and investigating, as appropriate, the reasons for them.

Correspondingly, Article 31 of the GDPR establishes the obligation of controllers and processors to cooperate with the supervisory authority that requests it in the performance of their functions. In cases where they have appointed a data protection officer, Article 39 of the GDPR assigns them the function of cooperating with that authority.

The favorable resolution of the Motion for Reconsideration filed, signed on June 23, 2023, determined the opening of the present procedure for the lack of attention to a request for the exercise of rights established in Articles 15 to 22 of the GDPR, regulated in Article 64.1 of the LOPDGDD, according to which:

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"When the procedure exclusively refers to the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of Regulation (EU) 2016/679 of the European Parliament and of the Council, of April 27, 2016, it shall be initiated by an admission agreement, which shall be adopted in accordance with the provisions of Article 65 of this organic law.

In this case, the deadline for resolving the procedure shall be six months from the date on which the admission agreement was notified to the claimant. After this period, the interested party may consider their claim accepted."

Article 58.2 of the GDPR grants the Spanish Agency for Data Protection a series of corrective powers for the purpose of remedying any infringement of the GDPR, including "ordering the data controller or processor to address the requests for the exercise of the rights of the data subject under this Regulation."

III

Rights of individuals regarding personal data protection

The rights of individuals regarding personal data protection are regulated in Articles 15 to 22 of the GDPR and Articles 13 to 18 of the LOPDGDD. The rights of access, rectification, erasure, objection, the right to restriction of processing, and the right to data portability are contemplated.

The formal aspects related to the exercise of these rights are established in Articles 12 of the GDPR and 12 of the LOPDGDD.

Additionally, consideration is given to what is expressed in Recitals 59 and following of the GDPR.

In accordance with the provisions of these regulations, the data controller must establish formulas and mechanisms to facilitate the exercise of rights by the data subject, which shall be free of charge (without prejudice to the provisions of Articles 12.5 and 15.3 of the GDPR), and is obliged to respond to requests made no later than one month, unless it can demonstrate that it is unable to identify the data subject, and to state its reasons in case it does not intend to comply with such request. The burden of proof for compliance with the duty to respond to the request for the exercise of rights made by the affected party lies with the data controller.

The communication addressed to the data subject regarding their request must be expressed in a concise, transparent, intelligible, and easily accessible manner, using clear and simple language.

IV

Right of access

In accordance with the provisions of Article 15 of the GDPR and Article 13 of the LOPDGDD, "the data subject has the right to obtain from the data controller confirmation as to whether or not personal data

concerning them is being processed and, if so, the right of access to the personal data."

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Regarding the right of access to personal data, in accordance with the provisions of Article 13 of the LOPDGDD, when the exercise of the right refers to a large amount of data, the data controller may request the affected party to specify the "data or processing activities to which the request refers." The right shall be deemed granted if the data controller provides remote access to the data, considering the request fulfilled (although the data subject may request the information referred to in the provisions of Article 15 of the GDPR).

The exercise of this right may be considered repetitive if it is exercised more than once within a period of six months, unless there is a legitimate reason for doing so. On the other hand, the request will be considered excessive when the affected party chooses a means other than the one offered that entails a disproportionate cost, which must be borne by the affected party.

Like the other rights of the data subject, the right of access is a personal right. It allows the citizen to obtain information about the processing of their data, the possibility of obtaining a copy of the personal data concerning them and being processed, as well as information, in particular, about the purposes of the processing, the categories of personal data concerned, the recipients or categories of recipients to whom the data may be communicated, the expected retention period or criteria for retention, the possibility of exercising other rights, the right to lodge a complaint with the supervisory authority, the information available about the source of the data (if it has not been obtained directly from the data subject), the existence of automated decisions, including profiling, and information about transfers of personal data to a third country or to an international organization. The possibility of obtaining a copy of the personal data being processed shall not adversely affect the rights and freedoms of others.

The right of access concerning medical records is specifically regulated in Article 18 of Law 41/2002, of November 14, which is the basic law regulating Patient Autonomy and Rights and Obligations in the Field of Information and Clinical Documentation (hereinafter LAP), the literal text of which states:

"1. The patient has the right of access, with the reservations indicated in section 3 of this article, to the documentation of the medical record and to obtain a copy of the data contained therein. Health centers shall regulate the procedure that guarantees the observance of these rights.

2. The patient's right of access to the medical record may also be exercised by duly accredited representation.

3. The patient's right of access to the documentation of the medical record cannot be exercised to the detriment of the right of third parties to the confidentiality of the data contained therein collected in the therapeutic interest of the patient, nor to the detriment of the rights of the professionals involved in its preparation, who may oppose the right of access based on the confidentiality of their subjective notes.

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4. Health centers and individually practicing professionals shall only provide access to the medical records of deceased patients to persons linked to them, for family reasons or, in fact, unless the deceased expressly prohibited it and this is accredited. In any case, access by a third party to the medical record motivated by a risk to their health shall be limited to the relevant data. Information affecting the privacy of the deceased or the subjective notes of the professionals, or that harms third parties, shall not be provided."

In this regard, it is important to highlight Article 15 of the LPA, which outlines the minimum content of the medical record:

"1. The medical record shall include the information deemed essential for the truthful and updated

knowledge of the patient's health status. Every patient or user has the right to have a record, in writing or in the most appropriate technical support, of the information obtained in all their care processes, carried out by the health service both in the primary care and specialized care settings.

2. The main purpose of the medical record shall be to facilitate healthcare, documenting all those data that, in the medical opinion, allow for the truthful and updated knowledge of the health status.

The minimum content of the medical record shall be as follows:

- a) Documentation related to the clinical-statistical sheet.
- b) Admission authorization.
- c) Emergency report.
- d) Anamnesis and physical examination.
- e) Evolution.
- f) Medical orders.
- g) Consultation sheet.
- h) Reports of complementary examinations.
- i) Informed consent.
- j) Anesthesia report.
- k) Operating room or childbirth record.
- l) Pathological anatomy report.
- m) Nursing care evolution and planning.
- n) Therapeutic application of nursing.
- ñ) Vital signs chart.
- o) Clinical discharge report.

Paragraphs b), c), i), j), k), l), ñ), and o) shall only be required in the completion of the medical record when it concerns hospitalization processes or as otherwise provided.

3. The completion of the medical record, in aspects related to direct patient care, shall be the responsibility of the professionals involved in it.

4. The medical record shall be maintained with criteria of unity and integration, in each healthcare institution at a minimum, to facilitate the best and most timely care."

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knowledge by the healthcare professionals of the data of a specific patient in each healthcare process."

Regarding the preservation of the medical record, Article 17 of the LPA, in its points 1 and 5, states that:

"1. Healthcare centers have the obligation to preserve clinical documentation under conditions that guarantee its proper maintenance and security, although not necessarily in the original format, for the appropriate assistance to the patient for the time required in each case and, at a minimum, five years from the date of discharge from each healthcare process (...)

5. Healthcare professionals who carry out their activity individually are responsible for the management and custody of the healthcare documentation they generate."

V

Conclusion

In the present case, from the examination of the documentation provided by the parties, it has been established that the claimant requested access to the medical record of his deceased wife, and that the respondent has sent him various documents. However, the claimant expresses his disagreement indicating that the medical record is incomplete, without the respondent having proven that they have delivered the entirety of it, or justified the denial of such delivery.

Consequently, it is appropriate to uphold the claim that led to this procedure.

In view of the cited provisions and others of general application, the Director of the Spanish Agency for

Data Protection RESOLVES:

FIRST: TO UPHOLD the claim made by Mr. A.A.A. considering that the provisions of Article 15 of the GDPR have been infringed and to urge the NAVARRO HEALTH SERVICE - OSASUNBIDEA with NIF Q3150004D, to send, within ten working days following the notification of this resolution, to the claimant a certification addressing the requested right or justifying the denial indicating the reasons why the request cannot be met, in accordance with the provisions established in the body of this resolution. The actions taken as a result of this Resolution must be communicated to this Agency within the same period. Non-compliance with this resolution could result in the commission of an infringement of Article 83.6 of the GDPR, classified as very serious for the purposes of prescription in Article 72.1.m) of the LOPDGDD, which will be sanctioned in accordance with Article 58.2 of the GDPR.

SECOND: NOTIFY this resolution to Mr. A.A.A. and to NAVARRO HEALTH SERVICE - OSASUNBIDEA.

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In accordance with the provisions of Article 50 of the LOPDGDD, this Resolution will be made public once it has been notified to the interested parties.

Against this resolution, which concludes the administrative procedure in accordance with Article 48.6 of the LOPDGDD, and in accordance with the provisions of Article 123 of the LPACAP, the interested parties may optionally file a motion for reconsideration before the Director of the Spanish Agency for Data Protection within one month from the day following the notification of this resolution or directly file an administrative contentious appeal before the Contentious-Administrative Chamber of the National Court, in accordance with the provisions of Article 25 and in section 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Contentious-Administrative Jurisdiction, within two months from the day following the notification of this act, as provided in Article 46.1 of the aforementioned Law.

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